



Republic of the Philippines
Province of Sorsogon
MUNICIPALITY OF PRIETO DIAZ
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Office of the Sangguniang Bayan

EXCERPT FROM THE MINUTES OF THE REGULAR SESSION OF THE 11TH SANGGUNIANBAYAN OF PRIETO DIAZ, SORSOGON HELD ON JUNE 3, 2024 AT ITS SESSION HALL

PRESENT:

Hon. Avon D. Domalaon, MPA	Municipal Vice Mayor/Presiding Officer
Hon. Jo Madel D. Domalaon	S. B. Member
Hon. Bienvenido D. Ayo, Jr.	-do-
Hon. Alice D. Emano	-do-
Hon. Juan D. Ebid	-do-
Hon. Albert D. Destura	-do-
Hon. Joseph H. Yap, Jr.	-do-
Hon. Antonio J. Emano, Jr.	LnB Pres. /S. B. Member
Hon. Rene Joseph D. Domasian	PPSK Pres./S.B. Member

ABSENT:

Hon. Luis Lee E. Espeña, Jr.	S.B. Member/ on leave
Hon. Rodolfo E. Diolata	-do-

ORDINANCE NO. 2024-09

Introduced by Hon. Alice D. Emano

AN ORDINANCE ESTABLISHING A COMPREHENSIVE, INTEGRATED, AND SUSTAINABLE POLICY AND COMMITMENT TO ELIMINATE TUBERCULOSIS (TB) IN THE MUNICIPALITY OF PRIETO DIAZ, SORSOGON AND APPROPRIATING FUNDS THEREOF

WHEREAS, the Local Government Code (LGC) of 1991, states that "every local government unit (LGU) shall exercise the powers expressly granted, implied, as well as powers necessary, appropriate, or incidental for efficient and effective governance. Under the general clause of the Code, the LGU shall ensure support in the promotion of health and safety of their constituents. The LGUs is likewise expected to be capable of responding to problems that include prioritization of health issues; monitoring of activities relative to health care; and adopting innovative and sustainable interventions for its constituents;

WHEREAS, Republic Act No. 10767 (TB Law) otherwise known as the Comprehensive Tuberculosis Elimination Plan Act, mandates the state to support and expand efforts to eliminate tuberculosis by 2035 by increasing investments for its prevention, treatment and control;

WHEREAS, while the Department of Health and the Provincial Health Office provide TB drugs, commodities and other supplies essential for program implementation, the municipality will allocate funds thereof to ensure continuity of quality TB services and address stock outs concerns as experienced from previous years;

WHEREAS, the enactment of a local TB policy or ordinance will ensure the adoption, and localization and implementation of effective, efficient and doable innovative and recommended strategies to eliminate TB;

NOW, THEREFORE, be it ordained, as it hereby ordained by the Sangguniang Bayan in session duly assembled:

SECTION 1. TITLE

This Ordinance shall be known as "TB Elimination Ordinance of the municipality of Prieto Diaz, Sorsogon."

SECTION 2. OBJECTIVE

This ordinance aims to institute, establish and localize a comprehensive, integrated, and sustainable LGU response and commitment towards TB elimination as espoused by the TB law and to help the country achieve the Philippine Strategic TB Elimination Plan (PhilSTEP) targets.

SECTION 3. DECLARATION OF POLICIES

It is hereby declared that the municipality of Prieto Diaz joins the national government in instituting health reforms to eliminate TB anchored on the mandates, provisions and recommendations of the NTP; TB Law and UHC. Equally important measures to be undertaken by the municipality to further to scale up TB elimination efforts include the following, to wit:

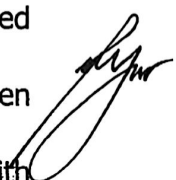
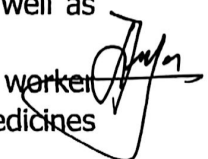
- a. Promote better public awareness and intensify community education on TB that includes but not limited to: TB as the top infectious killer disease in the world, leading killer of people with HIV and a major cause of deaths related to antimicrobial resistance; that 70 Filipinos die of TB every day; its modes of transmission; high risk groups; consequences of self-medication, non-adherence to DOTS and/or failure to complete treatment; control and prevention; and socio-economic impact;
- b. Develop an annual TB Plan with specific budget allocations based on needs assessment and the prevailing TB situation particularly on the following but not limited to TB medicines, laboratory reagents and supplies, human resource, capacity building, and ancillary medicines which will be incorporated in the Annual Operations Plan;
- c. Implement active case finding and reporting of all TB cases in the RHU, BHS, public hospital, and other health facilities handling TB patients as deemed necessary;
- d. Develop capacities of Human Resources for Health, Barangay Health Workers and Community Volunteers on trainings related to case-finding and case-holding to ensure that annual targets on the NTP are achieved;

- e. Establish a functional Primary Care Provider Network/ Health Care Provider Network as mandated by the UHC;
- f. Enforce the "No Prescription; No Dispensing" policy in all pharmacies to help mitigate the adverse consequences of self-medication such as the increasing microbial resistance or drug-resistant TB cases;
- g. Require compliance to the mandatory notification of all public and private healthcare providers and facilities with corresponding sanctions such as enforcement of penalties as provided for in the Revised IRR of RA 11332 Mandatory Reporting of Notifiable Diseases and Health Events of Public Health Concern Act;
- h. Ensure compliance to DOH AO 2015-0039-Guidelines on Managing Tuberculosis Control Program during Emergencies and Disasters and to provide support for NTP emergency/disaster preparedness and response; and
- i. Ensure DOTS Certification and PHIC accreditation of TB DOTS facilities as well as filing of PHIC TB DOTS package claims to further support the needs of the DOTS facilities and health staff involved in the TB program.



SECTION 4. DEFINITION OF TERMS. For purposes of this ordinance, the following terms shall be understood as follows:

- 1. **ACTIVE TB:** A person having TB with or without signs and symptoms, with bacteriologic and or radiographic findings consistent with TB disease
- 2. **ACTIVE TUBERCULOSIS CASE FINDING:** It is synonymous with systematic screening for active TB, although it normally implies screening that is implemented outside the health (DOTS) facilities
- 3. **CASE HOLDING:** An activity to treat TB cases through proper treatment regimen and health education
- 4. **CONTACT INVESTIGATION:** A systematic process for identifying people with previously undiagnosed TB among the contacts of an index case. The investigation includes identification of the source case if the index case is a child as well as candidates for preventive treatment
- 5. **DOT – Directly Observed Treatment:** An activity wherein a trained health worker for treatment partner personally observes the patient to take anti-TB medicines every day during the whole course of the treatment of all TB cases
- 6. **DOTS - Directly Observed Treatment Short-Course:** A comprehensive strategy to control TB comprised of five components
- 7. **DOTS FACILITY:** A health care facility, whether public or private, that provides TB-DOTS services in accordance with the policies and guidelines of the National TB Control Program (NTP), DOH
- 8. **DSSM: Direct Sputum Smear Microscopy.** Principal diagnostic method adopted by NTP because:
 - a. It provides a definitive diagnosis of active TB;
 - b. The procedure is simple;
 - c. It is economical; and
 - d. A microscopy center could be put up even in remote areas

9. DSTB (Drug Sensitive Tuberculosis)- is a case where a person is infected with TB bacteria that are susceptible to all the first line anti-TB drugs
10. DRTB (Drug Resistant Tuberculosis)- is a disease caused by bacteria that are spread from person through the air
11. INDEX (index patient) OF TB: The initially identified TB case of any age in a specific household or other comparable setting in which others may have been exposed
12. INTENSIFIED CASE FINDING: Systematic Screening in all health facilities among all consults
13. iDOTS: Integrated Delivery of Tuberculosis Services
14. IPC (Infection Prevention and Control)- is a practical, evidence-based approach preventing patients and health workers from being harmed by avoidable infections.
15. NTP (National TB Control Program)- is a program of the Department of Health which aims to reduce mortality and incidence from tuberculosis in the country, reduce catastrophic costs and provide patient-responsive health services
16. PMDT FACILITIES: Programmatic Management for Drug Resistant TB Facilities. A health (DOTS) facility that provide services for Drug Resistant TB
17. PRESUMPTIVE DRUG RESISTANT TB: Any person whether adult or child, who belongs to any of the DR-TB high-risk groups, such as: Re-treatment cases, new TB cases that are contacts of confirmed DRTB cases or non-converter of Category I, and people living with HIV with signs and symptoms of TB
18. PRESUMPTIVE TB: Any person whether adult or child with signs and/or symptoms suggestive of TB whether pulmonary or extra-pulmonary, or those with Chest X-ray findings suggestive of active TB
19. SYSTEMATIC SCREENING FOR ACTIVE TB: Refers to the systematic identification of people presumed to have active TB, in a predetermined target group, using tests, examinations or other procedures that can be applied rapidly.
20. TB: Tuberculosis. An infection caused by Mycobacterium tuberculosis.
21. TB TASK FORCE: A group of volunteers who will assist in most of the activities in the implementation of the TB program under the supervision of the TB Council
22. UHC (Universal Health Care) – is a health care system in which all residents of a particular country or region are assured access to health care.
23. Xpert/MTBRIF – is a new test that is revolutionizing tuberculosis (TB) control by contributing to the rapid diagnosis of TB disease and drug resistance

SECTION 5. TUBERCULOSIS CONTROL PROGRAM POLICIES

The Revised Manual of Procedures (MOP) 6th Edition for the National Tuberculosis Control Program (NTP), provides important guidelines for the effective and efficient implementation of the TB program. Therefore, all health care providers must abide and comply with all provisions embodied in the MOP.

1. Systematic screening shall be implemented in all DOTS (health) facilities. Cough of two weeks shall be the primary screening tool for systematic screening while Chest X-ray shall be done in targeted high-risk groups;
2. Active case finding shall be implemented in congregate settings such as jails and prisons, targeted community and workplace using chest x-ray as primary screening tool;

3. All People Living with HIV (PLHIV), diabetic patients, senior citizens, male smokers, household contacts of TB patients, indigenous people, and urban/rural poor are all encouraged to be screened for TB;
4. All health (DOTS) facilities should set up a strong TB surveillance amongst all employees and healthcare workers by providing free annual xray, free Rapid TB Diagnostic test, and DSSM;
5. Xpert MTB/RIF test shall be the primary diagnostic tool for diagnosis of both pulmonary and extra-pulmonary TB with or without high suspicion for multi-drug resistance. All presumptive pulmonary and extra-pulmonary TB shall be asked to expectorate a sputum sample and should undergo Xpert MTB/Rif test;
6. Other screening tests (i.e. Tuberculin Skin Testing-TST, diagnostic tests (i.e. Loop Mediated Isothermal Amplification-TB LAMP, Direct Sputum Smear Microscopy-DSSM, TB Culture) for TB shall also be used with or without Xpert MTB/Rif test if needed;
7. Direct Sputum Smear Microscopy (DSSM) shall be used for monitoring treatment of TB patients and in times where Rapid TB Diagnostic test is not available;
8. All health (DOTS) facilities, whether public or private shall establish their own in-house TB diagnostic laboratory (i.e. DSSM, Xpert MTB/Rif, Xpert Ultra and TB LAMP). All laboratories providing TB diagnostic tests, shall participate in External Quality Assurance (QA) System of the NTP Laboratory Network;
9. All diagnosed TB cases shall be provided with free adequate drugs and standard treatment for either drug susceptible or drug resistance TB regimen within 7 days from collection of sputum for diagnosis;
10. Adherence counselling shall be done for every patient prior to treatment. All lost to follow up TB patients and defaulters must be traced and referred back to treatment mobilizing all means available;
11. Fixed dose combination (FDC) composed shall be used as first line drugs (i.e. Isoniazid, Rifampicin, Pyrazinamide, Ethambutol) for drug susceptible TB while second line drugs (i.e. Quinolones, Bedaquiline, Delamanid, etc.) for drug resistant TB. For Latent TB Infection (LTBI), Isoniazid or Rifapentine and other available regimen shall be used among contacts of TB cases especially children and persons who are immunocompromised. Augmentation for TB medicines shall be provided for by the Local Government Unit.
12. Treatment adherence shall be ensured through patient-centered approaches. Treatment support shall be provided by health workers, community, or family members. All Adverse Drug Reactions (ADRs), whether minor or major, shall be reported using the official FDA reporting form. All registered TB patients fifteen years old and above shall be offered HIV Counselling and Testing (HCT);
13. Throughout the continuum of TB care, healthcare workers shall respect patient autonomy, and support self-efficacy. Patient physical comfort, safety, and wellness

shall be maximized with psycho-emotional support. The impact of poverty and food insecurity on TB diagnosis and treatment shall be recognized and addressed;

14. All baseline laboratories and other pertinent laboratories tests for DRTB during treatment and two years post-treatment shall be provided free whenever available in the municipality owned hospitals and Primary Care Facility;
15. Contact tracing and screening of all contacts of TB patients whether in household or workplace shall be done within 7 days;
16. All DOTS facilities and TB laboratories should always observe appropriate infection control measures following in order of hierarchy: administrative, environmental and respiratory controls;
17. Recording and reporting for the NTP shall be implemented at all DOTS facilities whether public or private according to internationally accepted case definition. NTP records should be kept for at least seven (7) years before properly discarding. The Integrated TB Information System (ITIS) shall be the official web-based electronic TB information system;
18. All Public and Private health care facilities shall designate a TB Notification Officer in charge of collection, consolidation, encoding in ITIS, and analysis of TB Notification reports and submission to the higher level.
19. All iDOTS facilities shall actively participate in the TB Medical Advisory Committee (TBMAC) to discuss difficult TB cases and management of adverse drug reactions to ensure case-holding and adherence to treatment of TB patients.
20. All DOTS facilities shall implement TB Preventive Treatment for all eligible contacts of TB patients and ensure availability of medicines with augmentation from the Local Government Unit.
21. All DOTS facilities shall implement an integrated approach to address the needs of patients including HIV and DM screening, and other health services as applicable

SECTION 6. CREATION AND COMPOSITION OF THE TUBERCULOSIS (TB) COUNCIL

6.1. The TB Council is hereby created to serve as an oversight body responsible in consolidating and harmonizing TB elimination efforts. The TB Council shall be composed of the following:

1. Chairperson: Municipal Mayor
2. Vice Chairperson: Municipal Health Officer

Members:

3. SB Chairman on Committee on Health
4. NGO / or GNIP Representative
5. NTP Nurse Coordinator
6. Liga ng Barangay President
7. DOH Representative
8. President, Barangay Health Workers Association

9. MSWD Officer
10. SKMF President
11. Chief of Hospital, PDMH
12. Municipal Nutrition Action Officer

6.2. The roles and functions of the TB Council include:

6.2.1 Identify and establish the roles and responsibilities of partners in the organization and delivery of quality TB services as per NTP guidelines.

- Ensure the socio-economic development policies and program and include consideration of the impact of TB infection to the community;
- Identify prioritization in the allocation of resources for the TB Program; and
- Spearhead activities and advocacy on TB events such as the Celebration of Lung Month and World TB Day.

6.2.2 Coordinate with the different sectors involved in the NTP implementation and ensure that the NTP policies and the DOTS strategy are implemented thereby ensuring case detection rate of at least 90% and treatment success rate of 90%.

- Strengthen partnership with other government agencies, NGOs and private entities and international agencies for a more comprehensive NTP implementation;
- Support local community health volunteers and other support group activities to sustain private sector interest and participation in the NTP; and
- Gather sources and additional support (financial and material) for the continuous implementation of the program.

6.2.3 Ensure that efforts and resources are generated and geared towards achieving the goal of having a community where TB is no longer a public health problem.

- Ensure that the collection for the budget requirements for the TB Program is sufficient;
- Ensure regular support for the monitoring, supervision, evaluation, training requirements, NTP drug and supplies;
- Advocate the continuous investment for quality improvement; and
- Ensure certification and accreditation of the health facilities as DOTS centers.

6.2.4 Create a TB Taskforce in the municipality (at least 1 volunteer per Barangay)

- Assist in all the activities of the Health Centers towards an efficient and effective implementation of the program;
- Help raise awareness and provide information campaign on TB during the house-to-house visits;
- Assist in data gathering, recording and monitoring of TB cases in the municipality; and
- Report regularly and work hand in hand with the barangay to ensure smooth implementation of the TB program.

6.2.5 Ensure that all pharmacies in the municipality shall be enjoined to enforce "*No Prescription; No Dispensing*" policy of TB Control for their implementation and compliance.

6.2.6 Ensure that infection control in all facilities and environment shall be implemented to prevent transmission among populations.

6.2.7 Ensure that all persons found to be presumptive TB are tracked and monitored until final diagnosis are achieved, that all diagnosed TB cases (i.e latent, DSTB, DRTB) are tracked and monitored until completion of their prescribed treatment regimens.

6.2.8 Adopt policies, guidelines and protocols of the NTP program

6.2.9 Prepare the Implementing Rules and Regulations (IRR) of this ordinance

6.2.10 Assign a TB Focal Person for the municipality

6.2.11 Hold a quarterly meeting

SECTION 7. MULTI-SECTORAL ALLIANCES

7.1. This will strengthen partnership with different sectors involved in the TB program such as government agencies, NGOs, civil society, private sector, donor institution and other cooperating agencies for a more comprehensive NTP implementation. All partner agencies shall contribute to address the needs of TB patients including education, livelihood, and other support mechanism to improve the lives of TB patients and the community as whole;

7.2. All public and private health facilities, hospitals, including pharmacies, private diagnostic clinics/centers, workplaces, transport groups, day care centers, schools and universities shall be engaged in TB control and prevention; and

7.3. All physicians practicing in the area shall have an orientation and update on TB to ensure key participation in TB Control and TB cases notified and reported to the RHU.

SECTION 8. TB AWARENESS/CAMPAIGN

8.1. A continuous promotion of TB awareness, and Active Case Finding and care shall be conducted in every barangay highlighted during the World TB Day (March 24) and the Lung Month (August 19) annually. This is in cooperation with all stakeholders/ development partners; and

8.2. The municipality shall provide logistical counterpart to all TB Awareness Campaign and caravan as a systematic screening activity among high risk community for TB such as the identified urban poor areas. Specifically, provisions for chest X-ray services and Xpert MTB/Rif test cartridges shall be supported.

SECTION 9. FUNDING ALLOCATION

The municipality of Prieto Diaz shall allocate three hundred thousand (P 300,000.00) annually, starting year 2025 and every year thereafter. The said fund shall be used for information education campaign and the procurement of medicines and other medical supplies for the treatment and elimination of tuberculosis in the municipality.

SECTION 10. REPEALING CLAUSE

All ordinances and resolutions inconsistent hereto are hereby modified, superseded, and repealed accordingly.

SECTION 11. SUPPLEMENTARY CLAUSE

On matter not provided in this ordinance, any existing applicable laws and their corresponding IRR, executive orders and relevant issuance therefore shall be applied in a supplemental manner.

SECTION 12: EFFECTIVITY CLAUSE

This Ordinance shall take effect upon compliance with posting requirement as provided in the Local Government Code (LGC) of 1991.

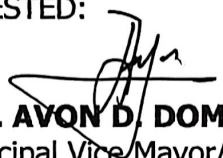
Let copies of this ordinance be forwarded to the Sangguniang Panlalawigan through Vice Governor Hon. Krunimar Antonio D. Escudero II, MDMG; copy furnished Hon. Romeo D. Domasian, Municipal Mayor; Dr. Jenifer Bernaldez, MHO, Municipal Health Office; Dr. Celso Lustestica, Chief of Hospital, Prieto Diaz Medicare Hospital; Ms. Gemma Ferrer, Municipal Treasurer; Ms. Maribel Aldon, Municipal Accountant; and all concerned for their information and appropriate action.

DATE ENACTED: JUNE 3, 2024

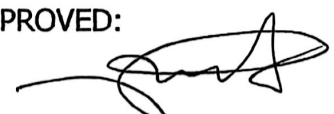
I HEREBY CERTIFY TO THE CORRECTNESS OF THE FOREGOING.


BERNADETTE R. ESPEÑA-YAO
Secretary to the Sangguniang Bayan

ATTESTED:


Hon. AVON D. DOMALAON, MPA
Municipal Vice Mayor/ Presiding Officer

APPROVED:


Hon. ROMEO D. DOMASIAN, PhD
Municipal Mayor
Date approved: JUN 25 2024



Republic of the Philippines
PROVINCE OF SORSOGON
Sorsogon City



SANGGUNIANG PANLALAWIGAN

SP Building, Gov. Augusto G. Ortiz Drive corner. Gov. Salvador C. Escudero, Sr. Drive,
Capitol Compound, Brgy. Burabod, East District, Sorsogon City

SEP 02 2024

August 6, 2024

TRANSMITTAL NO. 434-2024

TRANSMITTAL FOR: THE HONORABLE SANGGUNIANG BAYAN
Thru: Ms. BERNADETTE E. YAO
Secretary to the Sangguniang Bayan
LGU – Prieto Diaz, Sorsogon

Respectfully transmitting the herein resolution approved by the 11th Provincial Board on **July 22, 2024 (106th Regular Session)** for your **information and reference.**

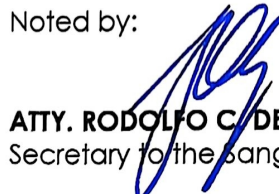
RESOLUTION NO. 349-2024

RESOLUTION DECLARING VALID PRIETO DIAZ ORDINANCE NO. 2024-09, ENTITLED: "AN ORDINANCE ESTABLISHING A COMPREHENSIVE, INTEGRATED, AND SUSTAINABLE POLICY AND COMMITMENT TO ELIMINATE TUBERCULOSIS (TB) IN THE MUNICIPALITY OF PRIETO DIAZ, SORSOGON AND APPROPRIATING FUNDS THEREOF."

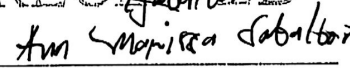
Prepared by:


RHEA T. DIESTA
Local Legislative Staff Officer III

Noted by:


ATTY. RODOLFO C. DEALCA, JR.
Secretary to the Sangguniang Panlalawigan

SB10-082924-2273

OFFICE of the SANGGUNIANG BAYAN LOCAL GOVERNMENT UNIT Prieto Diaz, Sorsogon
RECEIVED
BY: 
DATE: 8-29-24 TIME: 2:00 PM



Republic of the Philippines
PROVINCE OF SORSOGON
Sorsogon City



SANGGUNIAN PANLALAWIGAN

*SP Building Gov. Augusto G. Ortiz Drive corner Gov. Salvador C. Escudero, Sr. Drive,
Capitol Compound, Brgy. Burabod, East District, Sorsogon City-4700*

EXCERPT FROM THE JOURNAL OF THE 106th REGULAR SESSION OF THE 11TH PROVINCIAL BOARD ON JULY 22, 2024 AT THE SP SESSION HALL, SANGGUNIAN PANLALAWIGAN BUILDING, GOV. AUGUSTO G. ORTIZ DRIVE COR. GOV. SALVADOR C. ESCUDERO, SR. DRIVE, CAPITOL COMPOUND, BRGY. BURABOD, EAST DISTRICT, SORSOGON CITY.

PRESENT:

Hon. Krunimar Antonio D. Escudero II, Vice Governor/Presiding Officer

1ST DISTRICT

Hon. John Ryan C. Dioneda
Hon. Maria “Nini” O. Ravanilla
Hon. Ralph Walter R. Lubiano
Hon. Edmundo A. Atutubo
Hon. Rommel John C. Mella

2ND DISTRICT

Hon. Ramil M. Robles
Hon. Juan G. Guysayko
Hon. Benito L. Doma
Hon. Roland R. Afionuevo
Hon. Eduardo E. Ong Jr.

EX-OFFICIO

Hon. Ramon B. Escudero – Provincial President, Philippine Councilors League
Hon. Jose Arturo D. Enano – Provincial President, Liga ng mga Barangay
Hon. Karl Marx H. Lozada – Provincial President, Sangguniang Kabataan Federation

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RESOLUTION NO. 349-2024

RESOLUTION DECLARING VALID PRIETO DIAZ ORDINANCE NO. 2024-09, ENTITLED: “AN ORDINANCE ESTABLISHING A COMPREHENSIVE, INTEGRATED, AND SUSTAINABLE POLICY AND COMMITMENT TO ELIMINATE TUBERCULOSIS (TB) IN THE MUNICIPALITY OF PRIETO DIAZ, SORSOGON AND APPROPRIATING FUNDS THEREOF.”

WHEREAS, submitted to the Sangguniang Panlalawigan of Sorsogon was Prieto Diaz Ordinance No. 2024-09 for review and appropriate action;

WHEREAS, the Local Government of Prieto Diaz aims to institute, establish and localize a comprehensive, integrated and sustainable LGU response and commitment towards Tuberculosis (TB) elimination as espoused by the TB Law, and to help the country achieve the Philippine Strategic Tuberculosis Elimination Plan (PhilSTEP) targets. To achieve this, it is deemed important, and thus, recommended that an implementing guideline be instituted as well;

WHEREAS, the ordinance is *intra vires* as provided under Section 17 (b)(2)(iii) of R.A. No. 7160, otherwise known as the Local Government Code of 1991, considering that the LGUs are mandated to discharge the functions and responsibilities of national agencies and offices devolved to them, pursuant to this Code;

NOW, THEREFORE, on motion of Hon. Roland R. Añonuevo, duly seconded by Hon. Benito L. Doma, the Board **RESOLVED**, as it is hereby **DONE**, to **DECLARE VALID PRIETO DIAZ ORDINANCE NO. 2024-09, ENTITLED: "AN ORDINANCE ESTABLISHING A COMPREHENSIVE, INTEGRATED, AND SUSTAINABLE POLICY AND COMMITMENT TO ELIMINATE TUBERCULOSIS (TB) IN THE MUNICIPALITY OF PRIETO DIAZ, SORSOGON AND APPROPRIATING FUNDS THEREOF."**

LET COPIES of this resolution be furnished Mayor Romeo D. Domasian and the Sangguniang Bayan of Prieto Diaz, Sorsogon for their information and guidance.

APPROVED.

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I HEREBY CERTIFY to the correctness of the foregoing.


ATTY. RODOLFO C. DEALCA, JR.
Secretary to the Sangguniang Panlalawigan

ATTESTED:


HON. KRUNIMAR ANTONIO D. ESCUDERO II, MDMG
Vice Governor/Presiding Officer